

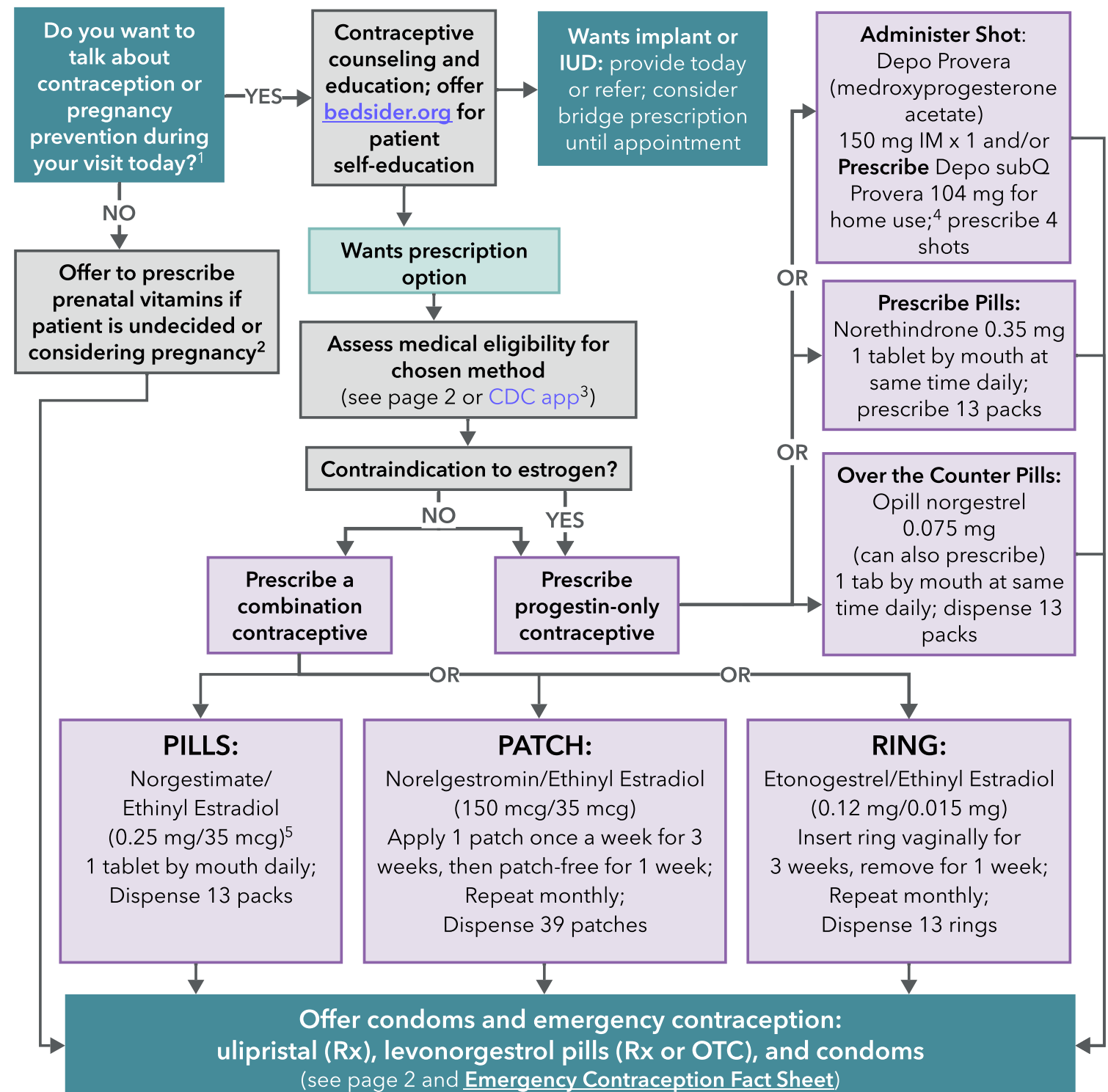
Quick Start Contraception Care in Any Setting

Health risks from pregnancy are always greater than the risks from any contraceptive method.

Not comprehensive: see [Bedsider.org](https://www.bedsider.org) for all options (including peri-coital and fertility awareness methods.)

June 2025

ACCESS TO CONTRACEPTION SAVES LIVES. Pregnancy tests, pelvic exams, labs, or consults not required.



1 Recommend for all patients with childbearing capability; could start with patients presenting with reproductive health concerns, depending on setting and capacity.

2 Folic acid prevents birth defects if taken early in pregnancy (see [CDC](https://www.cdc.gov)).

3 [CDC medical eligibility criteria](https://www.cdc.gov) for contraception.

4 See [PICCK guidelines](#) for Depo Provera subq for home use.

5 Many options available; this formulation is generic and well-tolerated.

See page 2 for disclaimer.

The Reproductive Health Hotline (1-844-REPROHH or 1-844-737-7644) www.reprohh.ucsf.edu

Free, confidential, nationwide, evidence-based hotline for health care providers. Monday-Friday 6am-5pm PT / 9am-8pm ET.

Benefits of Starting Contraception Immediately

June 2025

- Waiting for next period increases unplanned pregnancy rates by 30% (Zieman, et al., [Managing Contraception](#), 2024)
- Start contraception today if reasonably sure patient is not pregnant (CDC): is ≤ 7 days from start of menses, miscarriage, or abortion; is ≤ 4 weeks post-partum; has not had sex since last normal menses; is correctly using reliable contraception; or is fully/nearly exclusively breastfeeding, < 6 mo post-partum AND amenorrheic. (If patient does not meet criteria, use shared decision-making: contraception is not teratogenic; patient may choose to start anytime and do urine pregnancy test in 2 weeks.)
- Should prescribe/dispense emergency contraception (EC) also (see below).
- Should prescribe/dispense condoms.

Contraception Counseling and Education

- The best method is one that is safe and preferred by the patient.
- All hormonal methods reduce heavy bleeding and period cramping and have other non-contraceptive benefits.
- Irregular bleeding is common and expected with progestin-only methods.
- Depo Provera is the only method that has shown weight gain in studies. Depo Provera also has the longest return to fertility (up to 9 months).
- Bedsider (see QR code) has user-friendly method selection information including side-effect profiles. See [patient-centered counseling guide](#) for provider tips.

Scan for Bedsider
resources for
contraception
counseling



Medical Eligibility for Contraception

- **Avoid estrogen (combined methods)** with migraines with aura; chronic HTN; smoking ≥ 35 years old; venous thrombosis; pulmonary embolism; clotting disorder; stroke; active liver or gallbladder disease; Crohns or ulcerative colitis; complicated valvular heart disease; less than 30-42 days postpartum. Estrogen interacts with some anti-retrovirals, some anti-convulsants and rifampin.
- **Avoid estrogen and DMPA (Depo)** with sickle cell disease; 20+ year history of diabetes (or diabetes with complications and/or ischemic heart disease); severe cardiomyopathy or cardiovascular disease; decompensated cirrhosis; uncontrolled HTN; lupus with thrombocytopenia or antiphospholipid antibodies; and multiple risk factors for atherosclerosis.
- **Avoid all hormonal methods** for patients with current or past breast or liver cancer, severe kidney disease: can use copper IUD or barriers.
- **Avoid pill formulations** after bypass weight loss surgery: can use patches, rings, implants, IUDs, barriers.

Scan for CDC's
Medical Eligibility
Criteria for
Contraceptive Use



Providing Emergency Contraception (EC)

- IUD: the most effective EC; place up to 5 days after first unprotected intercourse or any time in cycle with negative urine pregnancy test.²
- EC can be used up to 3 - 5 days after intercourse and is more effective the sooner it is given.
- EC pills work by delaying ovulation. There are no medical contraindications to EC pill use.
- EC pills do not cause abortion,³ miscarriage, or birth defects.
- Side effects of EC pills are minimal; irregular bleeding and nausea are common.
- Wait 5 days after ella® before starting hormonal contraception or it could interfere with efficacy of ella®. Can use shared decision-making if same-day start of Depo or implant is best for patient.

Scan for
Emergency
Contraception
Fact Sheet



Providing a 13-Month Supply of Pills, Patches, or Rings

- Patients should be prescribed a 13-month supply of pills, patches, rings or Depo-Provera SQ.
- Running out of medication is the number one reason people stop taking oral contraception.
- 13-month prescribing reduces unintended pregnancy rates by 30% and abortion rates by 46% ([Foster 2011](#)).

More Information

- Most insurance plans cover multiple contraception methods at no cost.
- For uninsured patients, your state may have access programs for contraception; consider GoodRx for lower costs.
- Offer primary care follow up for preventive healthcare.
- All patients should be prescribed/provided emergency contraception and prenatal vitamins, if desired.

1 [Foster, et al](#). Number of oral contraceptive pill packages dispensed and subsequent unintended pregnancies. *Obstet Gynecol*. 2011 Mar;117(3):566-572. doi: 10.1097/AOG.0b013e3182056309. PMID: 21343759.

2 [BakenRa et al](#). Pregnancy Risk by Frequency and Timing of Unprotected Intercourse Before Intrauterine Device Placement for Emergency Contraception. *Obstet Gynecol*. 2021 Jul 1;138(1):79-84. <https://pubmed.ncbi.nlm.nih.gov/34259467/>.

3 [Winikoff, et al](#). New study with double-dose ulipristal for abortion requires misoprostol; **ulipristal alone is not an abortifacient**. A Proof-of-Concept Study of Ulipristal Acetate for Early Medication Abortion. *NEJM Evid*. 2025 Feb;4(2):EVIDoa2400209. doi: 10.1056/EVIDoa2400209. Epub 2025 Jan 23. PMID: 39847511.

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For additional Access Bridge resources, visit bridgetotreatment.org/access-bridge/.

Access Bridge is a program of the Public Health Institute's Bridge Center.